



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : هند عبدالودود اسماعيل مصطفى

المحافظة : القاهرة الحي / المدينة : السيده رينب

رقم البطاقة : 28103120100385 رقم التليفون : 01515451623 تاريخ تسجيل الإستمارة : 06-10-2026

العمر (تاريخ الميلاد) : 12-03-1981 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصيغة

[11129]

قسم العلاج :
الأورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **د. عبد الرزاق اسماعيل مصطفى**
National ID: **28103120100385**



تقرير طبي

المريض

تحتية طبية وبعد

بشأن تقرير طبي للمريض

إهداء للطلب المقدم من

رقم طبي:

رقم:

تشريف بأن توضح ليما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الشغل :
PM 01:17 05-07-2026

وبعد التشرف بالمحادثات والمتابعة وجدته

المرحلة الحالي من يوم غرضه والشيء و يحتاج الى Contrast-enhanced CT of chest, abdomen and pelvis.

توقيع :

د. عبد الرزاق

التاريخ :

05-07-2026

الطبيب المعالج :
الاستاذ الدكتور

جمهورية الكويت
وزارة الداخلية
إدارة أمن الشخصية



الجنس

عبدالودود اسماء هجرل مصطفى

* من عبدالله بن طاهر بكعة الكبيش

السيدة زينب - الطاهر *



٢ ٨١ ٠ ٣ ١٢ ٠ ١ ٠ ٠ ٣ ٨٥

LK6078808



٢٠٢٥/١٠ ٢٨١-٢١٢-١٠٠٣٨٤

حاصلته على دبلوم تجارة

التي من مملكة

الشرق محمد السيد الهادي

البلد من مملكة حتى ٢٠٢٢/١٠-٢٨١



Name: هيد عبد الودود اسماعيل مصطفى		
MR. : 8638		
Detailed Medical Report		
		السيد
تحت طية وبعد،،		
إيماء للطلب المقدم عن	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
يشترط بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمناظرة وجد أنه The patient presented with severe diarrhea, prompting a full laboratory work-up including CBC and electrolytes. The attending recommended consideration of a 25 % dose reduction for the next chemotherapy session. The patient underwent wide local excision (WLE) three weeks ago and is awaiting postoperative pathology results for c9 phesgo.		
تحريراً في ,		
التاريخ :	الوقت :	19-09-2026
الطبيب المعالج :	مايكل جمال فوري	
يعتمد ,		

Patient Name	هند عبد الوهيد اسماعيل	Case No.	343135
Sex/Age	Female 45 Y	Patient No.	25001
Registered Date	20/7/2026 12:03:01 PM	Referred By Dr.	Surgery
Validation Date	13/8/2026 11:43:43 AM	Branch	Gustave Roussy
Reporting Date	23/8/2026 03:54:04 PM	Pathology No.	2275/26

Pathology Report

Clinical Data: Left breast cancer, post-NACT.

Nature of the specimen: Left breast conservative surgery & Sentinel lymph nodes biopsy.

Gross Examination:

- * Left breast wide local excision specimen measured 10x05x05 cm, with covering skin ellipse measured 05x02 cm.
- * Serialling showed a single ill-defined mass measured 1.9x1.5x1.5 cm located 1.2 cm away from the deep margin.
- * Sentinel lymph nodes: Multiple fibro fatty tissue pieces measured 05x05 cm entangling 7 lymph nodes (only 6 were submitted for frozen sectioning), the largest node measured 1.0x1.0 cm.

* Frozen (Intra-operative) Results:

- Free surgical margins.
- SLNs: Six free sentinel lymph nodes (0/6), (Seven lymph nodes were detected within the sentinel lymph nodes specimen, only six were submitted for frozen & single one node for paraffin).

* Block designation:

* 9 Blocks:

1- Mass., 2- Medial margin., 3- Lateral margin., 4- Superior margin., 5- Inferior margin., 6- Deep margin., 7- Skin., 8,9- SLNs.

- Gross was done by: Dr. Aya Ezzat.

Microscopic examination:

- **Histological type:** Residual viable invasive duct carcinoma.
- **Tumor size:** 1.9x1.5 cm.
- **Cellularity:** 30%.
- **Grade:** 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- **Ductal carcinoma in situ:** Absent.
- **Lympho-vascular emboli:** Absent.
- **Perineural invasion:** Absent.
- **Resection Margins:** Free.
- **Sentinel lymph node involvement:** Seven sentinel lymph nodes free of viable metastatic deposits (0/7).
- **Histologic response to therapy:** Pathological Partial response to therapy.



طريقه رقم (8) - الطور 2 - مبنى رقم (1987) - جسر القسيه - القاهرة - مصر
 Bldg. No. (11) - First Floor Building No. 807- Nile Corniche - Mile El Quilwa - Cairo, Egypt

Patient Name	هند عبد الونود اسماعيل	Case No.	343135
Sex/Age	Female 45 Y	Patient No.	25001
Registered Date	20/7/2026 12:03:01 PM	Referred By Dr.	Surgery
Validation Date	13/8/2026 11:43:43 AM	Branch	Gustave Roussy
Reporting Date	23/8/2026 03:54:04 PM	Pathology No.	2275/26

Diagnosis:

Left breast cancer (post- NACT), Left breast conservative surgery & Sentinel lymph nodes biopsy:
 - Residual viable invasive duct carcinoma, grade 2, Cellularity 30% (Actual tumor size 1.9x1.5 cm).
 - Ductal carcinoma in situ: Absent.
 - Surgical margins: Free.
 - Sentinel lymph nodes: Seven sentinel lymph nodes free of viable metastatic deposits (0/7).
 Stage: ypT1c, ypN0.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.

Name:

Mrs. Hend Abdel-Wadood Ismaeil Mostafa

Doppler:

No.

Normal.

trivial.

trivial to mild.
18 mmHg.

No.

28 mmHg.
155 msec.

No.

Comments and conclusion:

- Good overall left ventricular systolic function, ejection fraction %65%.
- No apparent resting segmental wall motion abnormalities.
- Mild aortic valve sclerotic changes with trivial aortic regurge.
- Normal other valves structure and mobility.
- Normal intra-cardiac chambers dimensions.
- Estimated pulmonary artery systolic pressure around 28 mmHg.

Signature;

Dr. Hasan Ezz El-Din, MD

Figure 001: 200806 1000 PM

Request for Surgical Booking

Patient Name: Prof. Dr. J. C. G.
MRN: 50710.07

Patient's name			
Medical record number			
Date of birth			
Age & sex			
Name of surgeon			
Name of assistant			
Diagnosis			
Operation			
Expected duration of operation			
Date of operation		Time	
Type of surgery		☐ elective ☐ day care surgery Emergency: ☐ type I (life-threatening; to be done within 2 hours) ☐ type II (to be done within 6 hours) ☐ type III (to be done within 12-24 hours)	
Special instruments required to be ready before operation			
Requested by			
Date & time of the request			
Received in or by			
Date & time of receiving			
in case of cancellation			
Reason for cancellation			
Date & time for cancellation			
Canceled by			



Printed by: Cengage Learning India

Reviewed By:

CHEMISTRY UNIT

Patient Name:
 Patient No.: 278-906
 Accession No.: 27826071452
 Age: 45Y





Reviewed By:
 EV-Sally Staffs

Printed By: 07072026

HEMATOLOGY UNIT			
Complete Blood Picture (CBC)			
Test Name	Result	Unit	Reference Interval
Haemoglobin	12.10	g/dL	12 - 15
Hematocrit	36.5	%	36 - 46
Red cell count	4.88	$\times 10^9/\mu\text{L}$	3.8 - 4.8
MCV	74.9	fL	83 - 101
MCH	24.7	pg	27 - 32
MCHC	33.0	g/dL	31.5 - 34.5
RDW	19.0	%	11.6 - 14
Platelet Count	380	$\times 10^9/\mu\text{L}$	150 - 410
TLC	8.7	$\times 10^9/\mu\text{L}$	4 - 10
Differential Count			
	Percent Values	Absolute Values	
Basophils	1 %	0.1 $\times 10^9/\mu\text{L}$	0.02 - 0.1
Eosinophils	1 %	0.1 $\times 10^9/\mu\text{L}$	0.02 - 0.5
Stab	0 %	0.0 $\times 10^9/\mu\text{L}$	
Segmented	51 %	4.4 $\times 10^9/\mu\text{L}$	2 - 7
Lymphocytes	34 %	3.0 $\times 10^9/\mu\text{L}$	1 - 3
Monocytes	13 %	1.1 $\times 10^9/\mu\text{L}$	0.2 - 1
MPV	8.2 fL		8 - 12
Comments : Normal Hb, RBCs show mild microcytosis, erythrocytosis and moderate anisocytosis. Platelets are adequate. Mild absolute Monocytosis.			

Referring Doctor: 07072026
Review Date: 07/07/2026
Collection Date: 07/07/2026

Patient Name: 278-908
Accession No.: 27826071452
Age: 45Y





Reviewed By:
Dr. Sally Safar

Printed By: 001-00000000

HEMATOLOGY UNIT			
Test Name	Result	Unit	Reference Interval
Prothrombin Time and Conc.			
Control	12.4	Seconds	
Patient Time	11.0	Seconds	
Concentration	123	%	70 - 140
INR	0.89		Less Than 1
Partial Thromboplastin Time (PTT)			
Partial Thromboplastin Time	24.7	Seconds	24 - 36

Referring Doctor: 001-00000000
 Review Date: 07/07/2026
 Collection Date: 07/07/2026
 Patient Name: 001-00000000
 Patient No.: 278-906
 Accession No.: 27826071452
 Age: 45Y





Reviewed By:
for safety matter

Printed By: (طب) - (07/07/2026)

Test Name	Result	Reference Interval
HbS Ag, Serum	Negative	Negative
HCV Ab, Serum	Negative	Negative
HIV Ag-Ab, Serum	Negative	Negative

IMMUNOLOGY UNIT

Referring Doctor: (طب) - (07/07/2026)	Patient Name: (طب) - (07/07/2026)
Review Date: 07/07/2026	Patient No.: 278-906
Collection Date: 07/07/2026	Accession No.: 27826071452
	Age: 45Y



MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
SIGNATURE					

MDT OUTCOME & PLAN

Recommendations:
 for MDT to consider chemo regimen in light of poor response
 N/A (VTEH x6) - could start with abiraterone
 ✓

	BX: IDC CII	ER: 8/8	PR: 8/8	HER 2: 2+ (low)	Ki67: 25%
	Sonomammography: in majority of the quadrants →				
	Mets Workup: 10/15 - free				

Clinical Data					
Name	45 yr. - DH on Ind.				
	MRN	ID	Phone	Marital status	Menopausal status
			01100633451	married	premenopausal
Age	45 yr.				
Menopausal status	premenopausal				
NRS SCORE	N/A				
BMI	N/A				
CO morbidity	- DH on Ind.				

MDT FORM 5 / 2 / 2026 6



منظمة الرعاية الصحية الوطنية
 المملكة العربية السعودية



وزارة الصحة
 المملكة العربية السعودية

DR.

LV Dimensions
LV Wall Thickness
LV Diastolic Function
SWMA
LA and Ao Root Dimensions
Mitral Valve
Aortic Valve
Tricuspid Valve
Pulmonary Valve
Right Side and Systolic Function
Intracardiac Masses and Effusion

EF	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.9	AO.R	2.9	3-3.7cm
LVEDS		LA	3.8	2-4cm
FS		RV		0.7-2.3cm(supine)
SWT	1.0	AO.CUSP		1.5-2.6cm

NAME : Devi / 2021/2022
ID NUMBER : 307 / 2021
SEX : ♀
AGE : 30
DATE: 10 / 2 / 2026

ECHO REPORT

Name: Mrs. Hend Abd Al-Wadood Ismaeil Mostafa

Date: 11/7/2025

ID:2220034538

Acc:5275922

Referring Prof. Dr. Shaimaa Lashin. / مصلحة الجمارك والشؤون الطبية

Both axilla Ultrasonographic Examination, Revealed:

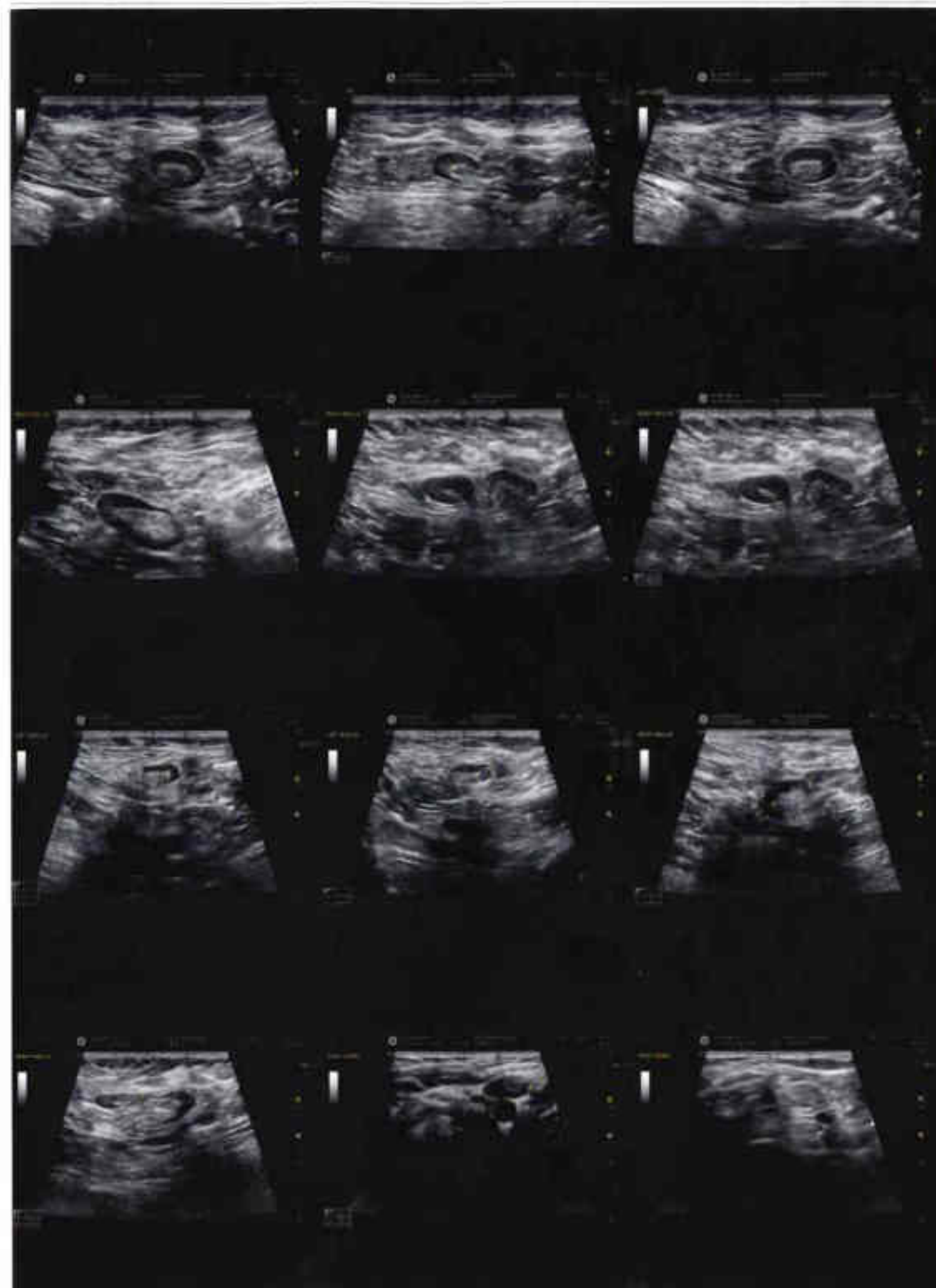
Indication: A known case of left breast cancer on chemotherapy.

Findings:

- Bilateral enlarged axillary lymph nodes (level I) with oval shape and central hilum, mildly thickened cortex up to 5 mm at right side and up to 3.8 mm at left side.
- No axillary lymph nodes enlargement at level II or level III.
- No evidence of other axillary cystic or suspicious solid lesions.
- Normal skin thickness and contour.
- Normal axillary vessels.

Truly yours
Dr. ASMA ARMANED, MD

Powered by IM-SoftLab v1.0.0.0.0.0




 جمهورية مصر العربية
 وزارة الصحة والسكان
 مبادرة رئيس الجمهورية
 لدعم صحة المرأة المصرية

08/31/2036 D

MDT FORM

Name: <u>د. عبد الرحمن محمد</u>	MRN: <u>8538</u>
Age: <u>45 Y</u>	ID: <u>28103130100385</u>
Menopausal Sta: <u>pre</u>	Phone: <u>01100633451</u>
NRS SCORE: <u></u>	Marital Status: <u></u>
BMI: <u></u>	Family History: <u></u>
CO Morbidity: <u>DM (uncontrolled)</u>	Reason Of MDT: <u>post op</u>

Clinical Data:

BX: IDC-GII
 ER: 8/8 PR: 8/8 HER2: 2+ amp Ki67: 25%

Sonomammography:**Meta Workup:****History:**

Private case
 Diagnosed Lt breast IDC cT2 N0 M0
 Received NACT (TCMP v6) ending 3 weeks ago
 Post-NACT mammosonography & Tomosynthesis...residual 27x20mm, with
 bilateral indeterminate axillary LFs.

SP LT WLE + SLNB (post op)
 residual visible invasive duct carcinoma G2
 free margins
 SLNB G17
 ypT1 ypN 0

for possibility of ad) tx

Recommendations:**MDT Outcome & Plan**

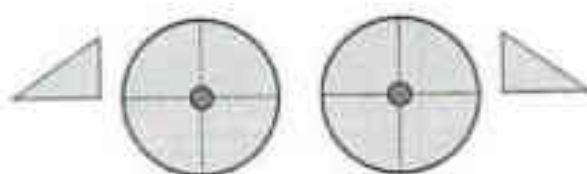
In view of non-PCR:
 1. PORT to Lt breast, CW & PCLN, VMAT
 2. Adjuvant Kadcyla x 14 cycle
 3. Adjuvant ET (AI) +/- zoledronic acid according to hormonal profile)
 4. Proin q 6 mths
 5. Endocrinology consultation for control of DM.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
<u>د. أحمد محمد</u>	<u>د. أحمد محمد</u>				

R

L



TNM

MDT FORM

Approved (and signed) by: *[Signature]* Date: 07/06/2016

MDT FORM

Name:	John Smith
Age:	45 Y
MRN:	28103120100385
Phone:	
Marital Status:	Married
Family History:	
Reason for Referral:	Post-MAC
MDT:	
CD:	CD (Unrecorded)
Morbidity:	

Clinical Data:

MDT Outcome & Plan

For target biopsy us then go for LT MLE & axillary management accordingly.

Recommendations:

Post-MAC mammography & Tumorboard - medical 27/2016, with bilateral

Diagnosed LT breast IDC K12 NO MO

Referred MACT (TCHP 16) ending 3 weeks ago

History:

PERCT scan free

Metastatic Workup:

Genomemicroarray:

ER: 8/8 PR: 8/8 HER: 2+ IN: K16J: 25%

DX: 10C 501

TNM

R

L

Medical Oncology	Surgery	Radiology	Pathology	Nutrition	Psychiatric

SIGNATURE

[Handwritten signatures and dates: 6/17/2016, 6/17/2016]

Dms / 8638

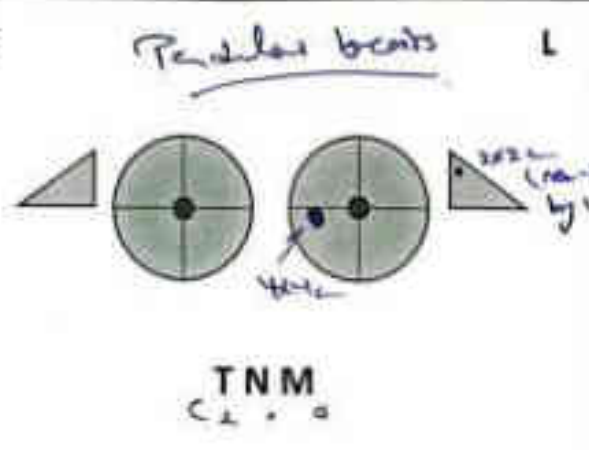
MDT FORM

5 / 2 / 2026

G

Name	50 / 2 / 2026	MRN	
Age	45 yr.	ID	
Menopausal status	permenopausal	Phone	01150633451
NRS SCORE		Marital status	married
BMI		Family history	ve.
CO morbidity	- DM on Insulin	REASON OF MDT	NAC

Clinical Data

BX:	IDC CIII			R	Tendous breast		L
ER:	8/8	PR:	8/8	HER 2:	2+ (low)	Ki67:	25%
Sonomammography:	Mammography of the breasts →						
Mets Workup:	PE/CT scan → free						

Recommendations:

For MDT to consider chemo treatment re stage
 NAC (TCHP x6) → consider adding a biologic
 if poor response

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

ECHO REPORT

NAME : مريم / مريم DATE: 10 / 2 / 2026
ID NUMBER : 507 / 1.07 SEX : ♀ AGE: 50

EF	<u>67%</u>	55-75%	PWT	<u>0.9</u>	0.6-1.1cm
LVEDD	<u>4.9</u>	3.5-5.8cm	AO.R	<u>2.9</u>	3-3.7cm
LVESD		2-4cm	LA	<u>3.8</u>	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	<u>1.0</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions normal
LV Wall Thickness normal
LV Diastolic Function DD grade I
SWMA normal
LA and Ao Root Dimensions normal
Mitral Valve normal TR
Aortic Valve normal
Tricuspid Valve normal TR normal (PASP)
Pulmonary Valve normal
Right Side and Systolic Function normal
Intracardiac Masses and Effusion normal

DR.

10/2/2026

جمهورية الكويت
وزارة الداخلية
إدارة أمن الشخصية



هــ

عبدالله اسماء هادي مصطفى

• من عبدالله بن طاهر بكعة الكبيش

السيدة زينب - القاهرة



٢ ٨١ ٠ ٣ ١٢ ٠ ١ ٠ ٠ ٣ ٨٥

LK6078808



٢٠٢٥/١٠ ٢٨١-٢١٢-١٠٠٣٨٤

حاصلة على ايام تجارة

التي مضافة

الشرك محمد السيد الهادي

البلد مصرية حتى ٢٠٢٢/١٠/٢٨



Patient Name	فد عبد الوهيد اسحاق	Case No.	343135
Sex/Age	Female 45 Y	Patient No.	25001
Registered Date	20/7/2026 12:03:01 PM	Referred By Dr.	Surgery
Validation Date	13/8/2026 11:43:43 AM	Branch	Gustave Roussy
Reporting Date	23/8/2026 03:54:04 PM	Pathology No.	2275/26

Pathology Report

Clinical Data: Left breast cancer, post-NACT.

Nature of the specimen: Left breast conservative surgery & Sentinel lymph nodes biopsy.

Gross Examination:

- Left breast wide local excision specimen measured 10x05x05 cm, with covering skin ellipse measured 05x02 cm.
- Seriatim showed a single ill-defined mass measured 1.9x1.5x1.5 cm located 1.2 cm away from the deep margin.
- Sentinel lymph nodes: Multiple fibro fatty tissue pieces measured 05x05 cm entangling 7 lymph nodes (only 6 were submitted for frozen sectioning), the largest node measured 1.0x1.0 cm.

• Frozen (Intra-operative) Results:

- Free surgical margins.
- SLNs: Six free sentinel lymph nodes (0/6), (Seven lymph nodes were detected within the sentinel lymph nodes specimen, only six were submitted for frozen & single one node for paraffin).

• Block designation:

- 9 Blocks.

1- Mast., 2- Medial margin., 3- Lateral margin., 4- Superior margin., 5- Inferior margin., 6- Deep margin., 7- Skin., 8, 9- SLNs.

- Gross was done by: Dr. Aya Ezzat.

Microscopic examination:

- **Histological type:** Residual viable invasive duct carcinoma.
- **Tumor size:** 1.9x1.5 cm.
- **Cellularity:** 30%.
- **Grade:** 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- **Ductal carcinoma in situ:** Absent.
- **Lympho-vascular emboli:** Absent.
- **Perineural invasion:** Absent.
- **Resection Margins:** Free.
- **Sentinel lymph node involvement:** Seven sentinel lymph nodes free of viable metastatic deposits (0/7).
- **Histologic response to therapy:** Pathological Partial response to therapy.



مركز جوستاف روبي
 Bldm 24, 91 - Paris France - Gustave Roussy Institute
 01 47 33 70 00 - www.gustaveroussy.fr

Name: Mrs. HEND Abdel-Wadood Ismaeil Mostafa

Date: 30/6/2026

ID: 2220034538

Acc.: 5271510

Referring: مصلحة الجوارك والغضاربي على قسطيات

M-Mode and 2D Findings:

M-Mode Measurements:

	Patient's values	Normal values
LV.	LVEDD	35-56 mm
	LVEDS	
	%F.S.	25-40%
	SWT	7-11 mm
	PWT	7-11 mm
R.V.	21	16-25 mm
L.A.	38	19-40 mm
Ao. Root	30	20-37 mm

- LV global systolic function: Good
- LV segmental wall motion abnormalities: No
- Inter-atrial septum: Intact
- Inter-ventricular septum: Intact
- Pericardium: Normal
- Intra-cavitary thrombi: No
- Mitral valve: Normal
- Aortic valve: mild sclerotic changes
- Pulmonary valve: Normal
- Tricuspid valve: Normal

Name: Mrs. Hend Abdel-Wadood Ismaeil Mostafa

Doppler:

Mitral flow:

Mitral incompetence:

No.

Max. diastolic P.G.:

Mean diastolic P.G.:

Mitral valve area:

E/A ratio:

Normal.

Aortic flow:

Aortic incompetence:

trivial.

Max. systolic P.G.:

Mean systolic P.G.:

Tricuspid flow:

Tricuspid incompetence:

trivial to mild.

Max. systolic P.G.:

18 mmHg.

Max. diastolic P.G.:

Mean diastolic P.G.:

Pulmonary flow:

Pulmonary incompetence:

No.

Max. systolic P.G.:

Mean systolic P.G.:

Pulmonary artery systolic P.:

28 mmHg.

Time to peak velocity:

155 msec.

Other abnormal flows:

No.

Comments and conclusion:

- Good overall left ventricular systolic function, ejection fraction %65%.
 - No apparent resting segmental wall motion abnormalities.
- Mild aortic valve sclerotic changes with trivial aortic regurge.
- Normal other valves structure and mobility.
- Normal intra-cardiac chambers dimensions.
- Estimated pulmonary artery systolic pressure around 28 mmHg.

Signature;

Dr. Hasan Ezz El-Din, MD

Typed: GHA. S. 2006/06/26 22:00:00

Request for Surgical Booking

Patient Name: *Substant*
MRN: *5071C.C7*

Patient's name			
Medical record number			
Date of birth			
Age & sex			
Name of surgeon			
Name of assistant			
Diagnosis			
Operation			
Expected duration of operation			
Date of operation		Time	
Type of surgery	☐ elective ☐ day care surgery	Emergency: ☐ type I (life-threatening: to be done within 2 hours) ☐ type II (to be done within 6 hours) ☐ type III (to be done within 12-24 hours)	
Special instruments required to be ready before operation			
Requested by			
Date & time of the request			
Received in or by			
Date & time of receiving			
In case of cancellation			
Reason for cancellation			
Date & time for cancellation			
Canceled by			



Lab 5

Collection Date: 07/07/2026

Review Date: 08/07/2026

Referring Doctor: د. محمد الحبيب

Patient Name: محمد الحبيب

Patient No.: 278-906

Accession No.: 27826071452

Age: 45Y

CHEMISTRY UNIT

Test Name	Result	Unit	Reference Interval
Glycated Haemoglobin (HbA1c), EDTA	11.8	%	< 5.7 Normal Range 5.7 - 6.4 Prediabetic ≥ 6.5 Diabetic
Plasma glucose assay is recommended monthly, HbA1C% and microalbumin/creol ratio once every three month, and lipid profile and eGFR to be monitored yearly in diabetics.			
ALT (SGPT), Serum	9	U/L	Less Than 50
AST (SGOT), Serum	8	U/L	Less Than 34
Urea, Serum	24	mg/dL	13 - 43
Creatinine, Serum	0.60	mg/dL	0.55 - 1.02



PL Branch: 053/44545 Ofc

Page 1 of 1

Reviewed By:

Dr. Safy Hafez

Printed by: 053/44545 Ofc 2024/7/25



Collection Date: 07/07/2026
Review Date: 07/07/2026
Referring Doctor: د. محمد عبد الله

Patient Name: محمد بن محمد بن محمد
Patient No.: 278-906
Accession No.: 27826071452
Age: 45Y

HEMATOLOGY UNIT

Complete Blood Picture (CBC)

Test Name	Result	Unit	Reference Interval
Haemoglobin	12.10	g/dL	12 - 15
Hematocrit	36.5	%	36 - 46
Red cell count	4.88	$\times 10^9 / \mu\text{L}$	3.8 - 4.8
MCV	74.9	fL	83 - 101
MCH	24.7	pg	27 - 32
MCHC	33.0	g/dL	31.5 - 34.5
RDW	19.0	%	11.6 - 14
Platelet Count	380	$\times 10^9 / \mu\text{L}$	150 - 410
TLC	8.7	$\times 10^9 / \mu\text{L}$	4 - 10

Differential Count	Percent Values	Absolute Values
Basophils	1 % 0 - 2	0.1 $\times 10^9 / \mu\text{L}$ 0.02 - 0.1
Eosinophils	1 % 1 - 6	0.1 $\times 10^9 / \mu\text{L}$ 0.02 - 0.5
Stab	0 % 0 - 7	0.0 $\times 10^9 / \mu\text{L}$
Segmented	51 % 40 - 80	4.4 $\times 10^9 / \mu\text{L}$ 2 - 7
Lymphocytes	34 % 20 - 40	3.0 $\times 10^9 / \mu\text{L}$ 1 - 3
Monocytes	13 % 2 - 10	1.1 $\times 10^9 / \mu\text{L}$ 0.2 - 1
MPV	8.2 fL 8 - 12	

Comments : Normal Hb, RBCs show mild microcytosis, hypochromia, erythrocytosis and moderate anisocytosis. Platelets are adequate. Mild absolute Monocytosis.



Print Barcode and Report
Page 1 of 1

Reviewed By:
Dr. Saad M. Al-Sayid

Printed By: Saad M. Al-Sayid 2026/07/07



1 2 3 4 5

Collection Date: 07/07/2026

Review Date: 07/07/2026

Referring Doctor: د. محمد عبد الله

Patient Name: محمد بن محمد بن محمد

Patient No.: 278-906

Accession No.: 27826071452

Age: 45Y

HEMATOLOGY UNIT

Test Name	Result	Unit	Reference Interval
Prothrombin Time and Conc.			
Control	12.4	Seconds	
Patient Time	11.0	Seconds	
Concentration	123	%	70 - 140
INR	0.89		Less Than 1
Partial Thromboplastin Time (PTT)			
Partial Thromboplastin Time	24.7	Seconds	24 - 36



Print Branch: 0000000000000000

Page 1 of 1

Reviewed By:

Dr. Saif Al-Sayid

Printed by: 0000000000000000

AME

A MEMBER OF ALFA MEDICAL GROUP

معمل اليفا

16191



Collection Date: 07/07/2026
Review Date: 07/07/2026
Referring Doctor: د. محمد الشاذلي

Patient Name: د. محمد الشاذلي
Patient No.: 278-906
Accession No.: 27626071452
Age: 45Y

IMMUNOLOGY UNIT

Test Name	Result	Reference Interval
HBs Ag, Serum	Negative	Negative
HCV Ab, Serum	Negative	Negative
HIV Ag-Ab, Serum	Negative	Negative



PL Branch: مستشفىalfa
Page 1 of 1

Reviewed By:
Dr. Safy Safy

Printed by: د. محمد الشاذلي

Diagnosis: DDC c2

Unit: _____

Diet Type: _____

Date	Time Of Order	Doctor's Order	Physician Name & ID	Signature	Nurse Name & ID	Signature	Time of Order Execution
		Left breast DDC Triplet positive					
		For c2 Tamoxifen / crushed pharynx					
		- vital signs					
		- creat : 0.54, lacto accepted					
		- Dexam + anal + kitayl on lacto saline					
		- Tamoxifen 25mg on Sun cc saline					
		- Carcin Her 5					
		- pharynx 600/600 SC					
		- Contrabac					
		- Methyl fline					
		- Calcium					
		- gymbrel					
		- aciclovir					
		- cefal					
		c3 25/13					
		c4 15/14					

* All orders, modifications, discontinuations, and corrections should be dated, timed, and signed.
* Do NOT use abbreviations.
* This form is NOT intended for use for modifications orders.

Transposed Mr. but in c. II
c¹ T₃ N₀ N₁
GC 8/R
AE 2/8

Diet Type:

100

Date	Time of Order	Doctor's Order	Physician Name & ID	Signature	Nurse Name & ID	Signature	Time of Order Execution
		use for anal & suprapubic pressure rectal or anal (incontinence) ITA 1-1-12				Winfred Heide/17 30/11	
		The growth started 1 month ago by arithmic away of 1 lb breast milk & increasing of 2 days before also usually. For this due to not be able to find me up to.					
		DE: 1 lb breast 9 o'clock from 455 r. Adm was the strong of 14 hours after 7-10 o'clock from 3000 84 x 25 x 28 cm					
		PERITONEAL: no detect metastases.					
		for: 1) the early 24 2) not a child with (the father & B. Anger) 3) both				Winfred Heide/17 30/11	
		The child was (ITC 170) (11/10/12/18) (Tanner 3-4) 9 1/2 years old for uric acid (E.g.)					

**This form is NOT intended for use for medications orders.*



Diagnosis: IBC G2

Unit: _____

Diet Type: _____

Date	Time or Order	Doctor's Order	Physician Name & ID	Signature	Nurse Name & ID	Signature	Time of Order Expiration
		let breast IDC G2 FR 8/8			PR 8/8	for 2 + 2 55h	2025/08/08
		for 2 Toxicity carb / pharynx					
		vital signs				Aliq min	
		labs					
		Dexa + anal + kidney					
		Tox test 75g (DD 25% for G2)					
		Carbon Act 5					
		Pharynx 1200/600 S.c					
		control				with Aliq min	30/11
		Methyl test		1x1x2f			
		calcium					
		onc alpha				3/13	
		catel				10/12/28	

* All orders, modifications, discontinuations, and corrections should be dated, timed, and signed.
* Do NOT use abbreviations.

* This form is NOT intended for use for medications orders.

Dms / 8638

MDT FORM

5 / 2 / 2026

G

Name	د. محمد عبد الله	MRN	
Age	45 yr.	ID	
Menopausal status	perimenopausal	Phone	01150633451
NRS SCORE		Marital status	married
BMI		Family history	none
CO morbidity	- DM on Insulin	REASON OF MDT	NACs

Clinical Data

BX:	TDC CII		
ER:	PR:	HER 2:	Ki67:
8/8	8/8	2+ (weak)	25%

Sonomammography:

homoechoic fibroadenoma →

Mets Workup:

PTA/CT scan - free

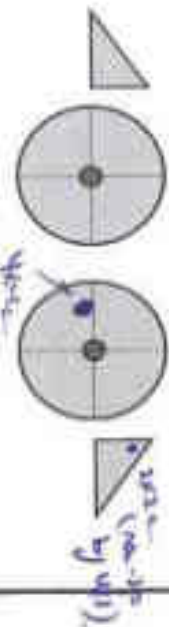
Recommendations:

For MDT to consider chemo option re staging
NACs (TDC x6) - consider adding abiraterone
if poor response

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



TNM
c2.0.0

GUSTAVE /
ROUSSY /
INTERNATIONAL
HOSPITAL

جوستاف
روسي
الطبيب
الدكتور

ECHO REPORT

NAME: د. عبد الله DATE: 10 / 2 / 2026
ID NUMBER: 507 / 001 SEX: ♀ AGE: 50

EF	<u>67%</u>	55-75%	PWT	<u>0.9</u>	0.6-1.1cm
LVEDD	<u>4.9</u>	3.5-5.8cm	AO.R	<u>2.9</u>	3-3.7cm
LVEDS		2-4cm	LA	<u>3.8</u>	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	<u>1.0</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR.

Chikio/2

Name: Mrs. Hend Abd Al-Wadood Ismaeil Mostafa

Date: 11/7/2025

ID:2220034538

Acc:5275922

Referring Prof. Dr. Shaimaa Lashin. / مصلحة الجمارك والشؤون الطبية

Both axilla Ultrasonographic Examination, Revealed:

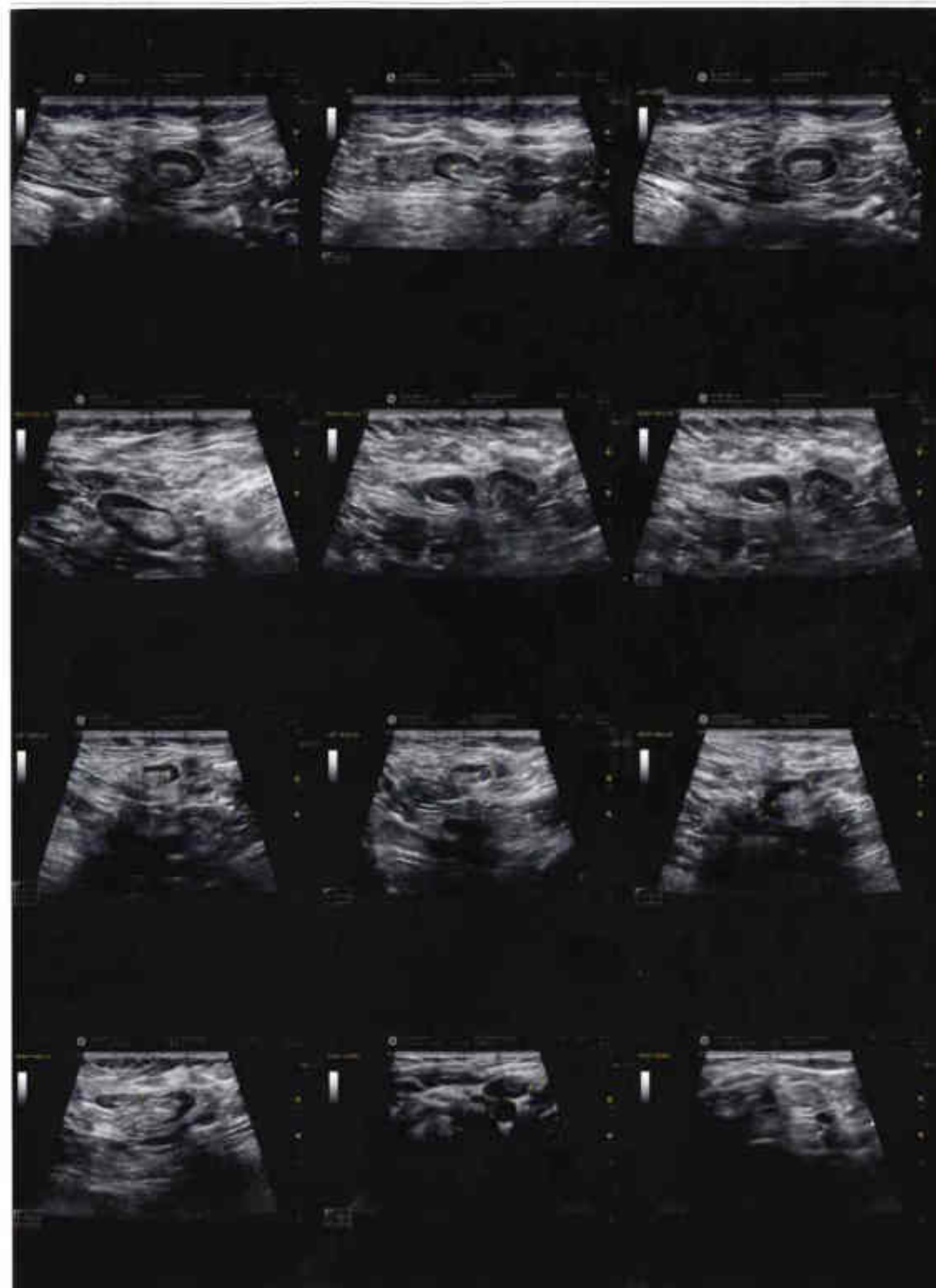
Indication: A known case of left breast cancer on chemotherapy.

Findings:

- Bilateral enlarged axillary lymph nodes (level I) with oval shape and central hilum, mildly thickened cortex up to 5 mm at right side and up to 3.8 mm at left side.
- No axillary lymph nodes enlargement at level II or level III.
- No evidence of other axillary cystic or suspicious solid lesions.
- Normal skin thickness and contour.
- Normal axillary vessels.

Truly yours
Dr. ASMA ARMANED, MD

Powered by IM-SoftLab v1.0.0.0.0.0



 جمهورية مصر العربية وزارة الصحة		مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية		 جمهورية مصر العربية	
MDT FORM					
NAME:	NAME:		NAME:		
AGE:	AGE:		AGE:		
Menopausal Sta:	Menopausal Sta:		Menopausal Sta:		
NRS SCORE:	NRS SCORE:		NRS SCORE:		
BM:	BM:		BM:		
CO Morbidity:	CO Morbidity:		CO Morbidity:		
Reason of MDT:		Reason of MDT:			

Clinical Data:

R

L

BX:	ICG III
EB:	EB: 2+ amp K1671 25%

Sonemammography

Mets Workup:

History:

Private case
 Diagnosed L1 breast IDC G2 G3 H0
 Received NACT (FOP x6) ending 3 weeks ago
 Post-NACT mammography & Tumour markers... Radical 27x20mm, with
 bilateral axillary lymphatic axillary LN.

SP CT WLE + SLNB (post op)
 residual viable invasive duct carcinoma G2
 free margins
 SLNB 0/7
 ypT3 ypN 0

for probability of +dG 12.

Recommendations:

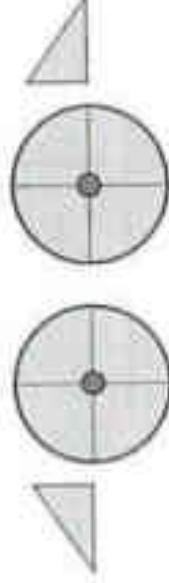
MDT Outcome & Plan

- In view of post-PCB:
1. Post to L1 breast, CW & PCN, VNAT
 2. Adjuvant Radiotherapy x 14 cycle
 3. Adjuvant ET (4) +/- zoledronic acid according to hormonal profile
 4. Prohormonal 4.5 mths
 5. Endocrinology consultation for control of DM.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	NUTRITION	PSYCHIATRY
Dr. H. H. H.	Dr. H. H. H.	Dr. H. H. H.	Dr. H. H. H.	Dr. H. H. H.	Dr. H. H. H.

Dr. H. H. H.



TNM

Page 1 of 1

8/13/2026

343135P3141

GUSTAVE
ROUSSY
Hôpital de la Pitié-Salpêtrière

Service de Pathologie Anatomique et Cytopathologique
Unité de Pathologie Anatomique et Cytopathologique

Dr. Y. Boug
Chirurgien

Patient Name	أنا عبد الوكيل السعيد	Case No.	343135
Sex/Age	Female 45 Y	Patient No.	25001
Registered Date	20/7/2026 12:03:01 PM	Referred By Dr.	Surgery
Validation Date	13/8/2026 11:43:43 AM	Branch	Gustave Roussy
Reporting Date	23/8/2026 03:54:04 PM	Pathology No.	2275/26

Pathology Report

Clinical Data: Left breast cancer, post-NACT.

Nature of the specimen: Left breast conservative surgery & Sentinel lymph nodes biopsy.

Gross Examination:

- * Left breast wide local excision specimen measured 10x05x05 cm, with covering skin ellipse measured 05x02 cm.
- * Scrolling showed a single ill-defined mass measured 1.9x1.5x1.5 cm located 1.2 cm away from the deep margin.
- * Sentinel lymph nodes: Multiple fibro fatty tissue pieces measured 05x05 cm entangling 7 lymph nodes (only 6 were submitted for frozen sectioning), the largest node measured 1.0x1.0 cm.

* Frozen (Intra-operative) Results:

- Free surgical margin.
- SLNs: Six free sentinel lymph nodes (0/6), (Seven lymph nodes were detected within the sentinel lymph nodes specimen, only six were submitted for frozen & single one node for paraffin).

* Block designation:

- * 9 Blocks:
- 1- Main., 2- Medial margin., 3- Lateral margin., 4- Superior margin., 5- Inferior margin., 6- Deep margin., 7- Skin., 8,9- SLNs.
- Gross was done by: Dr. Aya Ezziat.

Microscopic examination:

- * **Histological type:** Residual viable invasive duct carcinoma.
- * **Tumor size:** 1.9x1.5 cm.
- * **Cellularity:** 30%.
- * **Grade:** 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * **Ductal carcinoma in situ:** Absent.
- * **Lympho-vascular emboli:** Absent.
- * **Perineural invasion:** Absent.
- * **Resection Margins:** Free
- * **Sentinel lymph node involvement:** Seven sentinel lymph nodes free of viable metastatic deposits (0/7).
- * **Histologic response to therapy:** Pathological Partial response to therapy.

CAD

Dr. Y. Boug - Chirurgien
Boulevard Pasteur - 75014 Paris - France
Boulevard Pasteur - 75014 Paris - France
Boulevard Pasteur - 75014 Paris - France

Name: Mrs. Hend Abdel-Wadood Ismaeil Mostafa

Date: 1/9/2026

ID:2220034538

Acc.:5296020

Referring Prof. Dr. Jostaf Rosi / الدكتور راسي على الشعاعيات

M-Mode and 2D Findings:

M-Mode Measurements:

	Patient's values	Normal values
LV:		
LVEDD	50	35-56 mm
LVEDS	29	
%F.S.	41%	25-40%
SWT	11	7-11 mm
PWT	11	7-11 mm
R.V.	19	16-25 mm
L.A.	36	19-40 mm
Ao Root	34	20-37 mm

- LV global systolic function: Good
- LV segmental wall motion abnormalities: present
- Inter-atrial septum: Intact
- Inter-ventricular septum: Intact
- Pericardium: Normal
- Intra-cavitary thrombi: No
- Mitral valve: Normal
- Aortic valve: Normal
- Pulmonary valve: Normal
- Tricuspid valve: Normal

1. *What is the purpose of the study?*
 2. *What are the research objectives?*
 3. *What is the research design?*
 4. *What are the variables?*
 5. *What is the sample size?*
 6. *What are the data sources?*
 7. *What are the data collection methods?*
 8. *What are the data analysis methods?*
 9. *What are the results?*
 10. *What are the conclusions?*
 11. *What are the limitations?*
 12. *What are the recommendations?*

Name: HEND ABDELWADOOD ISMAEL MOUSTAFA
Date: 08 /02 /2026

ULTRASOUND GUIDED CLIPPING INSERTION

- Under local anesthesia, antiseptic condition and ultrasound guidance, 1 clip was done for the pathologically proved malignant left UIQ malignant mass .

Best regards,
Prof .Dr. MONA ZAKY
Breast Interventional Consultant

Name: Mrs. Hend Abd El-Wadood Ismaeil Mostafa

Date: 13/5/2026

ID:2220034538

Acc.:5254119

Referring Prof. Dr. Amr Mahmoud Abdel-Wahhab

Computed X-ray examination of the chest (PA view). revealed:

- Clear both lung fields and costophrenic recesses.
- Average cardiothoracic ratio.

Truly yours;

Dr. Esraa Mahmoud, M.Sc

Spelling checked by: Dr. en
Typed: GH. S. 13/05/2016

Issue No. (1)

Page 1 of 1

Code: F/TRC-OP-20-01

الطليحة - فرعها - (مركز الطليحة للتشخيص والتحاليل الطبية)
17 شارع 1 في شارع الطليحة فرعها - فرعها - فرعها - فرعها
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Name: Mrs. Hend Abdel-Wadood Ismaeil Mostafa

Date: 14/4/2026

ID:2220034538

Acc.:5242725

Refrerring: المصلحة الجمارك والتضرائب على المبيعات

M-Mode and 2D Findings:

M-Mode Measurements:

Mode Measurements:		Patient's values	Normal values
LV:	LVEDD	52	35-58 mm
	LVESD	32	
	%F.S.	38%	25-40%
	SWT	10	7-11 mm
	PWT	10	7-11 mm
R.V.		18	16-25 mm
L.A.		39	19-40 mm
Ao. Root		34	20-37 mm

- | | |
|---|--------|
| • LV global systolic function: | Good |
| • LV segmental wall motion abnormalities: | No |
| • Inter-atrial septum: | Intact |
| • Inter-ventricular septum: | Intact |
| • Pericardium: | Normal |
| • Intra-cavitary thrombi: | No |
| • Mitral valve: | Normal |
| • Aortic valve: | Normal |
| • Pulmonary valve: | Normal |
| • Tricuspid valve: | Normal |

Issue No. (1)

Page 1 of 2

Code: F/IRC-OP-20-01

د . هاني احمد سامي
دكتوراه الطب العامة والباطنية
استاذ بكلية طب القصر العيني



د . هاني علي الشيب
دكتوراه الطب العامة والباطنية
استاذ بكلية طب القصر العيني

Name:

Mrs. Hend Abdel-Wadood Ismaeil Mostafa

Doppler:

Mitral flow:

Mitral incompetence:

No.

Max. diastolic P.G.:

Mean diastolic P.G.:

Mitral valve area:

E/A ratio:

reversed.

Aortic flow:

Aortic incompetence:

No.

Max. systolic P.G.:

Mean systolic P.G.:

Tricuspid flow:

Tricuspid incompetence:

trivial to mild.
17 mmHg.

Max. systolic P.G.:

Mean diastolic P.G.:

Pulmonary flow:

Pulmonary incompetence:

No.

Max. systolic P.G.:

Mean systolic P.G.:

Pulmonary artery systolic P.:

Time to peak velocity:

27 mmHg.
145 msec.

Other abnormal flows:

No.

Comments and conclusion:

- Good overall left ventricular systolic function, **ejection fraction %65%.**
 - No apparent resting segmental wall motion abnormalities.
 - Impaired diastolic relaxation (Reversed E/A ratio).
- Normal valves structure and mobility.
- Normal intra-cardiac chambers dimensions.
- Estimated pulmonary artery systolic pressure around 27 mmHg.

Typical ECG 5/14/2018 13:10 PM

Signature:
Dr. Hasan Ezz El-Din, MD

Issue No. (1)

Page 2 of 2

Code: F/ITRC-OP-20-01

Name: Mrs. Hend Abdel-Wadood Ismaeil

Date: 8/1/2026

Referring / مصلحة الجمارك والضرائب على المبيعات

ID:2220034538

Acc.:5213487

Digital 3D soft tissue mammography & tomosynthesis with ultrasound examination of both breasts, revealed:

Indication: left breast palpable lump, positive family history.

Mammography & Tomosynthesis:

- Scattered fibroglandular densities (ACR-b)
- **Left breast:** inner central, middle third irregular shaped spiculated margin, heterogeneous dense lesion showing internal clustered / grouped microcalcifications about 5.5 cm distance from the nipple.
- **Right breast:**
 - No evidences of dense spiculated masses or architecture distortion seen.
 - No suspicious micro-calcific clusters noted.
- Normal skin thickness and contour of both sides.
- Normal areola / nipple complexes on both sides.

Ultrasonographic examination showed:

- Heterogeneous fibroglandular breast parenchyma with predominant fatty tissue.
- **Left breast:** inner central 9-10 o'clock irregular shaped spiculated margin heterogeneous hypoechoic solid lesion surrounded by echogenic rim measuring 2.5 x 3.4 x 2.3 cm in average diameter, 0.8 cm deep to skin, posterior acoustic shadowing and prominent internal vascularity of low resistance arterial waveform pattern and associated with skin retraction.
- **Right breast:** No gross evidence of suspicious cystic or solid masses.
- No retro-areolar dilated ducts noted.
- Normal skin thickness and contour of the right side.
- Normal nipple / areola complexes.
- Bilateral non-specific axillary lymph nodes with preserved oval shaped and central echogenic hilum.

Opinion:

- Right breast: Normal study (BIRADS 2).
- Left breast: 9-10 o'clock highly suspicious lesion as described (BIRADS 4c).

Recommendation:

Ultrasound guided biopsy from the left breast highly suspicious lesion.

Dr. Lamia Elsharkawy, MD

BIRADS is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions.

• **BIRADS 0:** Incomplete, need additional imaging evaluation or comparison with previous films.

• **BIRADS 1:** Normal examination, nothing to comment on. Routine mammography screening is recommended.

• **BIRADS 2:** Benign findings, routine mammography screening is recommended.

• **BIRADS 3:** Probably benign <2% malignancy, short interval follow up is suggested.

• **BIRADS 4:** Suspicious >2 but <95% - biopsy should be considered.

• **BIRADS 5:** High suspicion of malignancy >95% - appropriate action should be taken.

• **BIRADS 6:** Known biopsy proven malignancy. No need for additional imaging.

Assessment of the breast density: Mammography and ultrasound should be taken.

• **M.B.1:** scattered areas of fibroglandular density.

• **M.B.2:** heterogeneously dense.

• **M.B.3:** extremely dense.

Name	Mrs. Hend Abdel-Wadoud Ismail Mostafa		
Ref. By Prof. Dr.	Ehab Saad Abden-Nabi	Date	28/1/2026
ID	2220034538	Acc. No.	291311566

2220034538

¹⁸F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of left breast malignancy. No available previous studies or pathological data.

Total body PET scan was done 45 minutes after IV administration of 370 MBq of ¹⁸F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROIs. The normal threshold would be the normal blood pool liver SUV max = 2.2.

Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain. (N.B; not all brain metastatic lesions could be detected on ¹⁸-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-pharynx, larynx, thyroid and salivary glands.
- No enlarged FDG hypermetabolic cervical L.Ns seen.

Torso:

Thorax:

- *Low activity left breast upper domain lesion about 1.5 cm of SUVmax. 1.2.*
- *Small low activity bilateral axillary lymph nodes, largest measuring about 8 mm of SUVmax. 1; however, no enlarged FDG hypermetabolic mammary related lymph nodes.*
- *For mammographic correlation.*
- Bilateral tracer starved atelectasis and sub-pleural reticulations.
- No FDG hypermetabolic pulmonary nodules or masses seen.
- No enlarged FDG hypermetabolic hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.
- Hiatus hernia.

Abdomen & Pelvis:

- Average size of the liver with non-uniform activity yet no FDG hypermetabolic focal lesions, mostly blood pooling. For dynamic MRI study of the liver assessment if warranted.
- No biliary tree dilatation.

Name	Mrs. Hend Abdel-Wadoud Ismail Mostafa		
Ref. By Prof. Dr.	Ehab Saad Abden-Nabi	Date	28/1/2026
ID	2220034538	Acc. No.	291311566

- Average size of the spleen with no focal active lesions.
- Normal appearance of pancreas (no sizable active masses), adrenals and pelvic organs (bulky uterus with low activity right adnexal cyst about 4 cm of SUVmax. 1.5; mostly physiologic, for USG correlation).
- No enlarged FDG hypermetabolic abdominal or pelvic L.Ns seen.
- Congested mesentery with strandy reticulations.
- Normal physiological biodistribution over the bowel (non-uniform bowel uptake with no sizable masses) & urinary tracts.

Surveyed skeleton:

- Faint marrow uptake of SUVmax. 2.8 recorded at L5 yet no tracer active lytic or sclerotic osseous lesions seen, likely hematopoietic, for CBC correlation and follow up.
- Scattered sclerotic non-metabolically active foci of the spine and pelvic bones (mostly degenerative changes).

Opinion:

- Low activity left breast lesion, for detailed mammographic correlation.
- Rest of the surveyed body is free from any other FDG hypermetabolic neoplastic masses.

Thanks for your referral;

Dr. Ahmed Kandeel

MD Nuclear Medicine

Dr. Islam Aboulwafa, MD

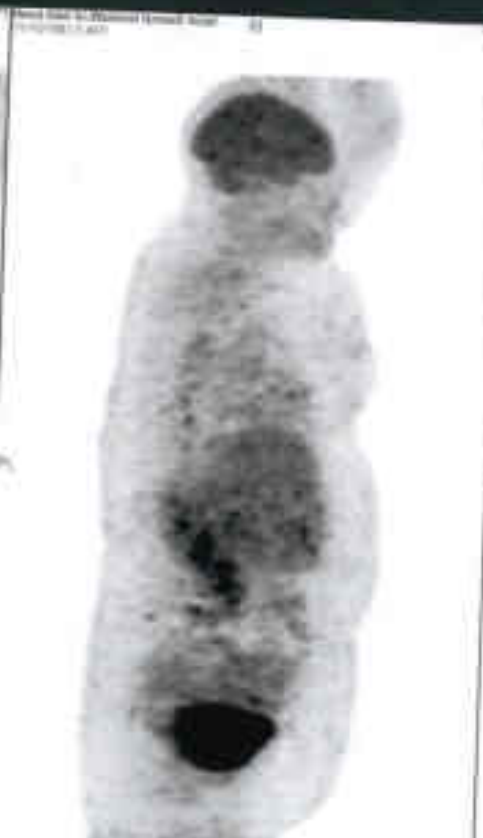
Consultant Radiologist

Hend Abd Al-Wadood Ismaeil most
2220034538

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Hend Abd Al-W... F 44Y

STUDY 1/28/2026

Ref: Abd Elnabi, Ehab. Saad
28/01/2026



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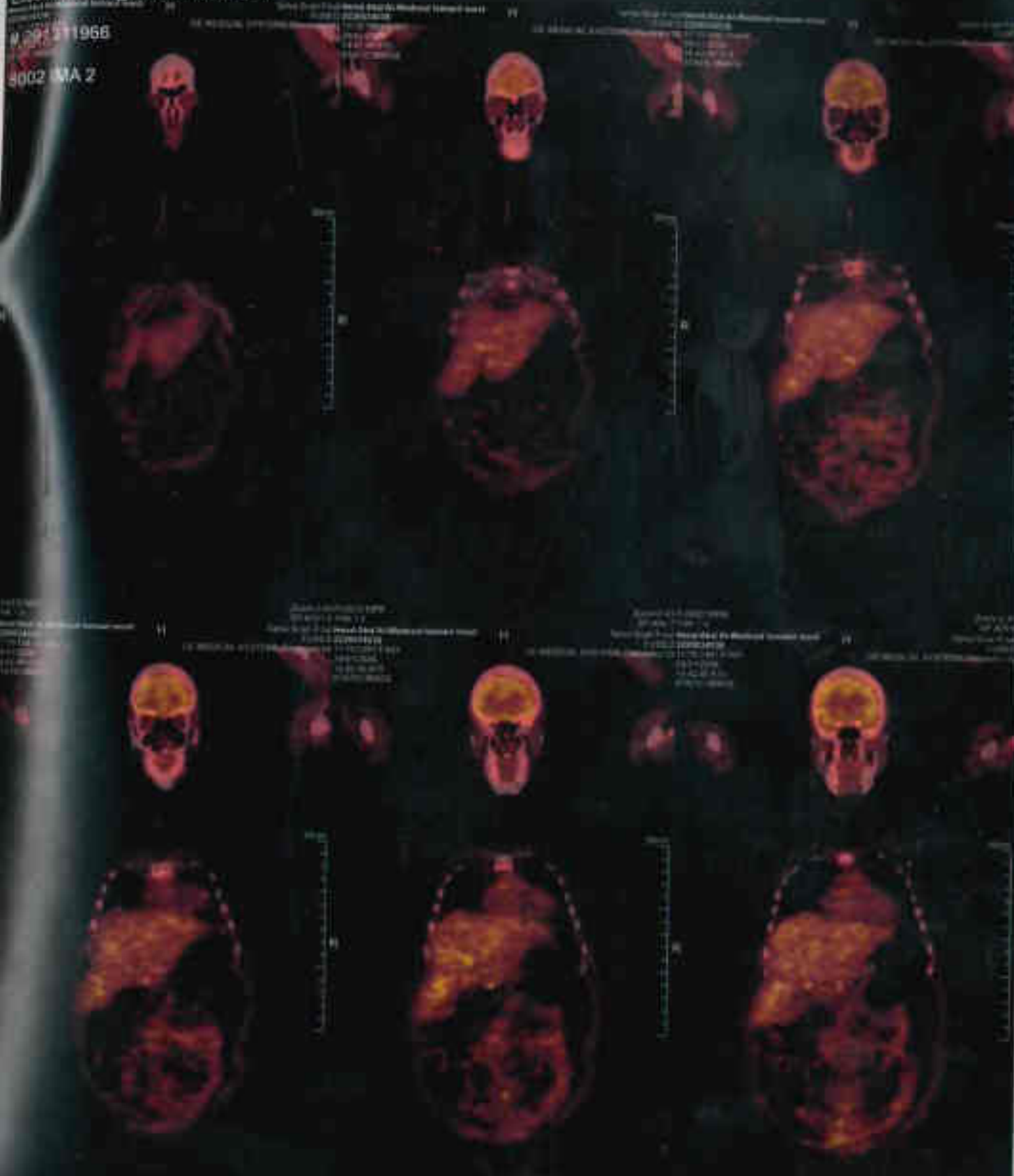
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STUDY 0/26/2026

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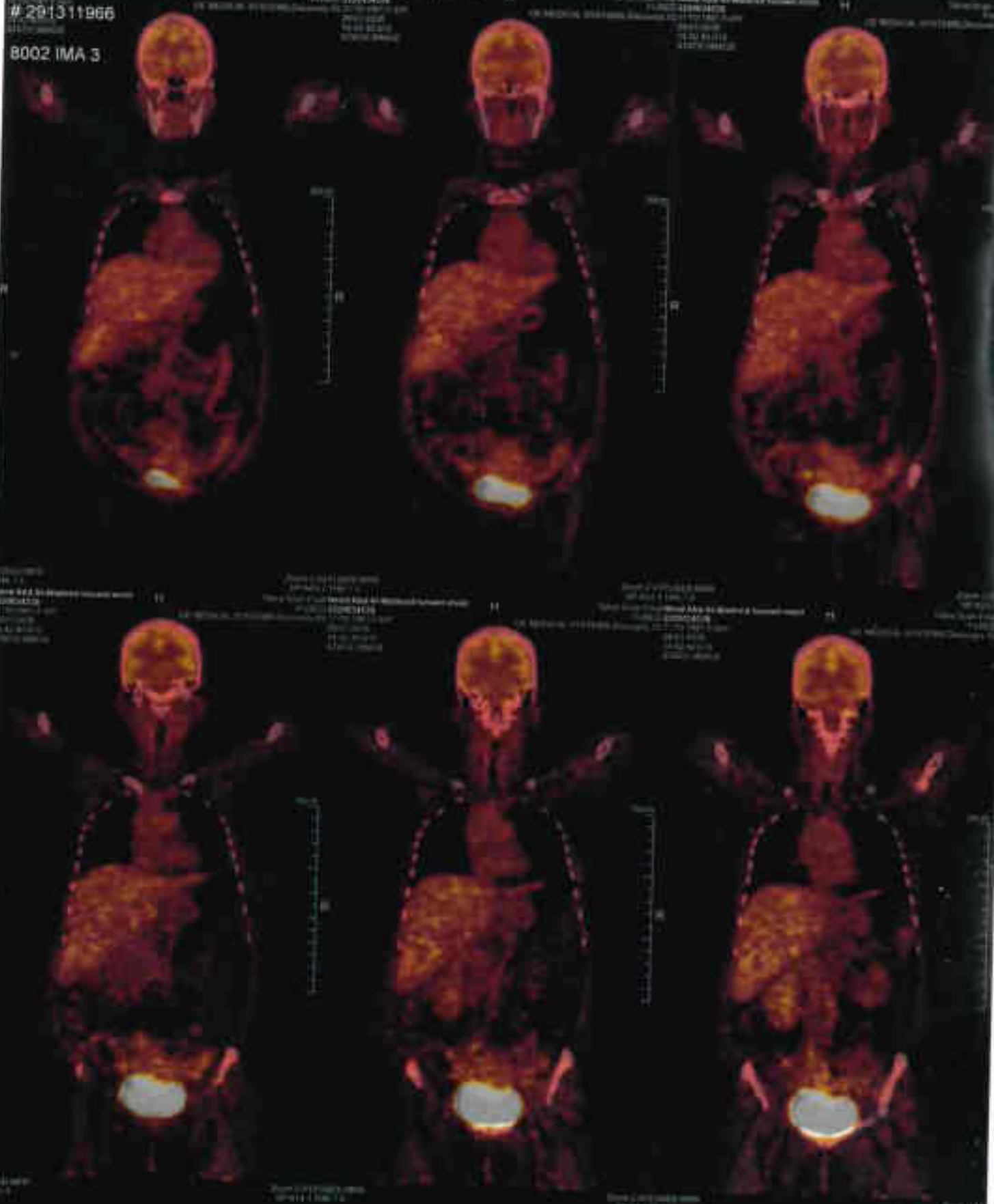
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18F-FDG Tahra Scan Fisa

28/01/2026

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Ref: Abd Elnabi, Ehab, saad

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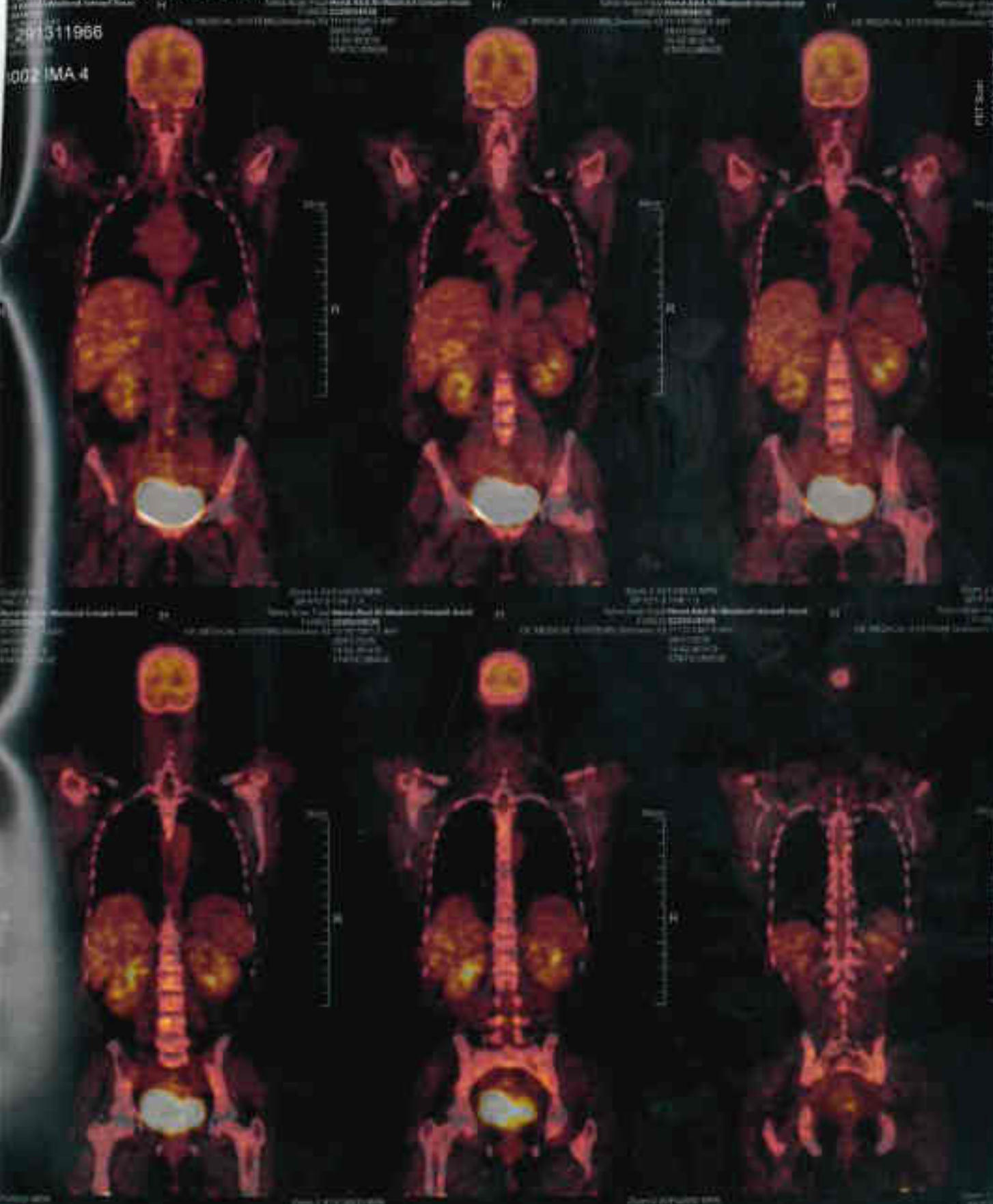
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18F-FDG Tahra Scan Fisa

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Tahra Scan Fisa
Ref: Abd Elnabi, Ehab, saad

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Name: Mrs. Hend Abdel-Wadood Ismaeil Mostafa

Date: 14/4/2026

ID:2220034538

Acc.:5242723

Referring / مصلحة الجمارك والطيران على المبيعات

Digital 3D soft tissue mammography & tomosynthesis with ultrasound examination of both breasts, revealed:

Indication: known case of pathologically proved malignancy, receiving chemotherapy, the current study is for post-therapeutic assessment

Mammography & Tomosynthesis:

- Scattered fibroglandular densities (ACR-b).
- **Left breast:** inner central, middle third irregular shaped spiculated margin, heterogeneous dense lesion showing internal clustered / grouped microcalcifications measuring 2.5 x 1.8 x 3 cm about 6.5 cm distance from the nipple.
- **Right breast:**
 - No evidences of dense spiculated masses or architecture distortion seen.
 - No suspicious micro-calcific clusters noted.
- Normal skin thickness and contour of both sides.
- Normal areola / nipple complexes on both sides.

Ultrasonographic examination showed:

- Heterogeneous fibroglandular breast parenchyma with predominant fatty tissue
- **Left breast:** inner central 9-10 o'clock irregular shaped spiculated margin heterogeneous hypoechoic solid lesion surrounded by echogenic rind measuring 23 x 21 x 19 mm, 11 mm deep to skin (compared to 2.5 x 3.4 x 2.8 cm in the previous study) posterior acoustic shadowing, no significant internal vascularity in complementary color Doppler study

Name: Mrs. Hend Abdel-Wadood Ismaeil Mostafa

- **Right breast:** No gross evidence of suspicious cystic or solid masses.
- Bilateral retro-areolar dilated ducts noted up to 0.3 cm with clear anechoic content.
- Normal skin thickness and contour. Normal nipple/areola complexes.
- Bilateral axillary lymph nodes with preserved oval shaped and central echogenic hilum (cortical thickness up to 4 mm in both sides) mostly reactive.

Opinion:

- **Right breast:** duct ectasia (**BIRADS 2**).
- **Left breast:** known case of pathologically proved malignant lesion showing regressive course regarding size, echogenicity and vascularity compared to the previous study dated 8th January 2026 (**BIRADS 6**).

Dr. Lamia Elsharkawy, MD

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BI-RADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- BI-RADS 0: Incomplete - need additional imaging evaluation or comparison with previous films.
- BI-RADS 1: Normal examination - nothing to comment on - Routine mammography screening is recommended.
- BI-RADS 2: Benign findings - routine mammography screening is recommended.
- BI-RADS 3: Probably benign <2% malignancy - short interval follow-up is suggested.
- BI-RADS 4: Suspicious >2 but <95% - Biopsy should be considered.
 - 4A: low suspicion of malignancy: 2-20%.
 - 4B: Mod suspicion of malignancy: 10-50%.
 - 4C: High suspicion of malignancy: 50-90%.
- BI-RADS 5: Highly suspicious of malignancy >95% - appropriate action should be taken.
- BI-RADS 6: Known - biopsy proven malignancy. Surgical excision when clinically appropriate.

American college of radiology (ACR) scoring of breast density:

- ACR a: almost entirely fatty.
- ACR b: scattered areas of fibro-glandular density.
- ACR c: heterogeneously dense.
- ACR d: extremely dense.